جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : مروه عبدالفتاح عبدالرحمن احمد
المحافظة : القاهره **الحي / المدينة :** البساتين **العنوان :** 1ش حسن السيد
رقم البطاقة : 27902170102521 **رقم التليفون :** 01113099290 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 17-02-1979 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مسح ذري علي العظام [12966] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: مروه عبدالفتاح عبدالرحمن احمد
National ID: 27902170102521

GUSTAVE ROUSSY INTERNATIONAL EGYPT

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Bone scintigraphy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



IDMS

ايبرانش



National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

تابع: مروة عبد الفتاح عبد الرحمن

- *The liver is of average size with homogenous CT texture. No other focal lesions or contour irregularities.*
- *Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.*
- *No intra-hepatic or extra-hepatic biliary radicles dilatation.*
- *Normal size & parenchymal texture of the spleen with no focal lesions seen.*
- *Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.*
- *Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.*
- *Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.*
- *Normal appearance of the other pelvic organs.*
- *No abdominal or pelvic masses or collections.*

Opinion:

Follow up study showing :

- ***Stationary course of the previously noted multiple sclerotic lesions scattered throughout the vertebra , ribs , sternum and pelvic bones , most likely osseous metastatic deposits.***
- ***Newly recognized hepatic dome small focal lesion , its enhancement pattern matching with hepatic hemangioma for follow up .***
- ***Left thyroid lobe heterogenous nodule for neck U/S assessment .***

DR.Ahmed ELdeeb,MD

CT Scan
[[DOCTYPE:CT_SCAN]]

Name: مروه عبدالفتاح عبدالرحمن احمد	
MR. : 3583	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient was noted to have breast cancer and was placed on a regimen of Ibrance, Aromasin, and Zoladex. A comparative CT performed on 21/12/2025 versus 4/3/2024 demonstrated multiple bony sclerotic lesions on bone scan. A subsequent bone scan on 25/1/2026 (DOKI scan) showed no definitive evidence of osseous metastatic disease. The plan was to continue Aromasin while awaiting new approval for Ibrance.

The patient was reported to have a local recurrence of breast cancer. The treatment plan continued to include Ibrance (pending) and Aromasin.

The patient was continued on Aromasin and Ibrance with a therapeutic drug level (TLC) of 3.04/1.19. Pharmacy dispensing was scheduled for 31/5, and Zoladex was continued.

The patient received Aromasin only, as Ibrance was dispensed on 20/5 pending a new CT scan for further assessment.

Laboratory studies were within acceptable limits for continuation of Ibrance 125 mg and Zoladex 10.8 mg. A follow-up CT scan was planned in one month.

The patient remained on Aromasin as of 8/7/2026. Pharmacy records indicated a CT scan on 18/6/2026, which showed stable osseous lesions and a newly recognized small hepatic dome lesion whose imaging characteristics were consistent with a hemangioma; further follow-up was recommended.

Laboratory results were accepted for continuation of Ibrance 125 mg and Aromasin on 8/8, with pharmacy processing confirmed.

Laboratory results were accepted for continuation of Ibrance 125 mg and Aromasin on 10/9, with pharmacy processing. The patient was scheduled for a neck duplex ultrasound to assess a potential arteriovenous catheter, as the patient requested its removal; the clinical impression noted right axillary lymph nodes for further evaluation.

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : هنا اسامه عبد الرحيم عبد البر

يعتمد ,

Fatth Islamic Center

C.T. - M.R.I

U.S. - Duppler - E.C.G. Stress

E.E.G - E.M.G - Mamm - Eye Unit

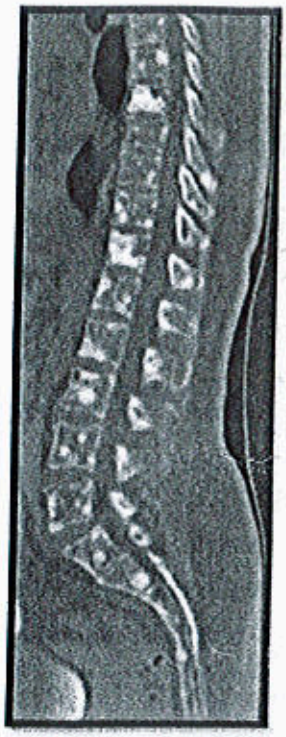
الجمعية الشرعية

مركز الفتح الإسلامى

أشعة مقطعية بالكمبيوتر - الرنين المغناطيسى
موجات صوتية - دوبلر ملون - رسم قلب بالمجهود
رسم مخ - رسم عضلات - فحص ثدى - وحدة عيون

اسم المريض: مروة عبد الفتاح عبد الرحمن التاريخ: ٢٠٢٦/٦/١٨

MULTI-SLICE CT SCAN OF THE Chest , ABDOMEN AND PELVIS AFTER BOTH ORAL & I.V. CONTRAST ADMINISTRATION: -



- **Status :Follow up study of known case of cancer breast .**
- **Compared to previous CT study dated 21/12/2025 the current study revealed :**

I- *Chest Findings:*

- *Evidence of left mastectomy with clear operative bed*
- ***Rather stationary course** of the previously noted multiple sclerotic lesions scattered throughout the vertebra , ribs , sternum and pelvic bones . no notable structure collapse or para-osseous soft tissue component.*
- *In situ porta cath is also seen .*
- *Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.*
- *No hilar or mediastinal lymph nodes.*
- *No evidence of pleural effusion.*
- *No gross cardiac abnormality. No pericardial effusion detected.*
- *Normal CT appearance of the thoracic aorta and the great vessels.*
- *The lower neck cuts show thyroid nodule involving the left lobe of thyroid gland measuring bout 20x23 mm at maximum axial dimensions . no retro sternal extension .*



II- Abdomen and Pelvis Findings:

- *Newly recognized hepatic dome small focal lesion eliciting peripheral nodular enhancement at arterial phase with progressive centripetal fill-in at Porto-venous and delayed study phases (no washout) . This lesion measuring about 5x8 mm .*

T.B.C.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name:	*Marwa Abd El-Fattah Abd*	*Patient ID:*	*635828*
		Exam Date:	*25/1/2026*

^{99m}Tc-MDP whole body bone scan

Desired information:

- Skeletal surveying

Technique:

- Anterior and posterior Delayed Whole body bone scan performed 2 hours after I.V administration of 20 mCi 99mTc-MDP:

Report

Delayed static phase:

- Compared to previous bone scan dated 22/6/2024, the current study revealed still stationary course of the degenerative changes at the dorsal &lumbar vertebrae, more pronounced at mid and lower dorsal.
- Arthritic changes at both shoulder and knee joints.
- Still seen facet joint arthropathy at LV5.
- There is no hot spot that account for osseous deposits could be detected allover the surveyed skeleton.
- Both kidneys are visualized.

Conclusion:

- ***No definite evidence of metastatic osseous deposits or serious bone lesions.***

Best regards;
Dr. Huda Fathy M.D.

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Patient Name:	*Marwa Abd El-Fattah Abd El-Rahman*	*Patient ID:*	*635828*
		Exam Date:	*22/6/2024*

^{99m}Tc-MDP whole body bone scan

Skeletal surveying

Anterior and posterior Delayed Whole body bone scan performed 2 hours after I.V administration of 20 mCi 99mTc-MDP:

Report

Compared to previous bone scan dated 16/1/2024, the current study revealed rather stationary course of the degenerative changes at the dorsal &lumbar vertebrae, more pronounced at mid and lower dorsal.

Arthritic changes at both shoulder and knee joints.

Still seen facet joint arthropathy at L.V5.

There is no hot spot that account for osseous deposits could be detected allover the surveyed skeleton.

Both kidneys are visualized.

Conclusion:

- ***Follow up bone scan shows rather stationary course with still no definite evidence of metastatic osseous deposits or serious bone lesions.***
- ***Spondylo- degenerative changes at the spine.***

Best regards;
Dr. Huda Fathy M.D.

Bone Scan
[[DOCTYPE:BONE_SCAN]]

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U.S. - Duppler - E.C.G. Stress
E.E.G - E.M.G - Mamm - Eye Unit

الجمعية الشرعية
مركز الفتح الإسلامى
أشعة مقطعية بالكمبيوتر - الرنين المغناطيسى
موجات صوتية - دوبلر ملون - رسم قلب بالمجهود
رسم مخ - رسم عضلات - فحص ثدى - وحدة عيون

اسم المريض : مروة عبد الفتاح عبد الرحمن التاريخ : 2025 / 12 / 21

C.T. REPORT

Multislice CT scan examination was done for the Chest, Abdomen & Pelvis
Multiplanner reconstruction images were obtained *following* I.V. contrast enhancement and gut opacification with Telebrix
with special *triphasic* helical scanning of the liver

In comparison with the last study dated 4-3-2024, the followings are noted

I - Chest:

- *Multiple ribs, vertebral bodies and pelvic bones sclerotic lesions are seen in the current study with no para osseous soft tissue components could be seen .*
- *Evidence of left mastectomy.*
- *Prominent pulmonary parenchymal vessels with basal atelectatic bands are once again noted.*
- *Otherwise, no other pulmonary abnormality could be detected.*
- *The pleural recesses are clear.*
- *The chest wall is preserved*
- *No mediastinal mass lesion or gross lymphadenopathy could be detected.*

II - Abdomen & Pelvis:

- *The liver is of normal size and regular outline, with no obvious focal mass lesion or dilated biliary radicle.*
- *The spleen, gall bladder and retroperitoneal structures including the kidneys, adrenal glands and pancreas are within normal limits, with no obvious mass lesion or significant lymph node enlargement.*
- *The opacified bowel loops including the stomach are regularly filled, with no obvious mass lesion or significant mural thickening.*
- *No ascites or pelvic collections seen.*
- *Normal C.T. appearance of the urinary bladder and uterus with IUD is seen.*

Impression:

- ***Follow up study of known case of cancer breast underwent left mastectomy showing multiple bony sclerotic lesions in the current study for further isotope scan if the possibility of metastasis has to be further evaluated.***
- ***Low grade pulmonary inflammatory reaction.***

YOURS SINCERELY,
Prof. Dr. Mohammed Fouad,
PROF. OF RADIO-DIAGNOSIS
EL-AZHAR UNIVERSITY

Hs-GH

٥٣ شارع ١٠ بالمعادى ت : ٢٣٥٨٧٣٣٢ - ٢٣٥٨٥٨١٩ - ٠١١٠٠١٥١٣٨٠ - ٠١٠٢٨٤٨٣٦٩٥

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

Patient Name: Marwa Abd Al Fattah Abd Al Rahman Ahmed	Date : 27/02/2025
Exam : C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 3118/2022	Pat. ID: 16757

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:

Compared to last non contrast study 16/10/2024

Findings:

I Chest:

- Evidence of left MRM. Clear operative bed with no masses or collection.
- No sizable pulmonary masses, consolidation or cavitation.
- **Still noted** Minimal left upper lung lobe fine subpleural reticulations, possibly post-radiation sequel.
- Diffuse mosaic attenuation of the lung parenchyma.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Right port-a-cath with its tip reaching the right atrium.
- Left thyroid lobe hypodense nodule for ultrasonography correlation.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture currently appreciated segment VIII dense focal lesion measuring 8 mm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.

CT Scan
[[DOCTYPE:CT_SCAN]]

MDT form

A-Patient ID:
1- Name : [redacted] Sex.
2-ID number: 27502170102521
3- Medical number: 01097567
Female
Old Med :
Patient Age : 46 Y
3116/2025

Family history	Phone	01113099255	Dr/		REASON OF MDT
marital status	NRS Score		GROUP		Date of MDT
Co morbidity	Menopausal status		Initial STAGE		2025-05-29

Clinical data

Breast Imaging :

Mets Workup :

BX	ER	PR	Her 2 Neu	SISH	Ki67
ILC	8/8	0/8	2	Not Amplified	10

T	N	M
4b	1	

Sonomammiographs
LT BREAST MASS(10-2 O'CLOCK)
IPSILATERAL AXILLARY LN ENLARMENT(BIRADS 4B)

PATIENT HISTORY
PT RECEIVED 12 WKS TAXOL+ 4 CYCLES AC WITH GOOD CLINICAL RESPONSE BUT STILL RESIDUAL MASS 5/4 CM
- LT MRM: 3/2
FREE AREOLA & NIPPLE
- FREE DEEP MARGIN
POSITIVE METASTATIC DEPOSITS IN 8 LN 8/10
PARTIAL PATHOLOGICAL RESPONSE
STAGE: YPT2 , YPN2, started adjuvant nolvadex
1/2024: irresectable chest wall local recurrence
1st line ibrance+AI
bone scan:free
last CT : Stable
O/E : COMPLETE clinical response regarding chest wall nodules
mri->hemangioma
pt off teratment since 2/2025 waiting for MRI liver, now reappreance of chest wall nodule

MDT OUTCOME & PLAN

MRI ABDOMEN : HEMANGIOMA
PATIENT IS OFF TTT SINCE FEBRUARY 2025
NEW CTCAP BASELINE
START FASLODEX,IBRANCE I ZOMETA/6MONTHS

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	

MDT Form
[[DOCTYPE:MDT_FORM]]

Fatih Islamic Center
C.T. - M.R.I
U.S. - Duppler - E.C.G. Stress
E.E.G - E.M.G - Mamm - Eye Unit

الجمعية الشرعية
مركز الفتح الإسلامي

اسم المريض : مروه عبد الفتاح عبد الرحمن التاريخ : ٢٠٢٥/٥/٦

M.R.I. REPORT

DYNAMIC M.R.I. OF THE ABDOMEN

Technique:
Multiple pulse sequences in different planes before and after IV contrast.

Findings:



- *Minimally enlarged liver showing a small lobulated focal lesion at hepatic dome (segment VIII) measuring about 1X0.9cm displaying low T1 and bright T2/IR signal. In the post-contrast series, such focal lesion showed peripheral nodular post-contrast enhancement at arterio-portal phase with more deposition of contrast within its core at delayed phase of the study.*
- *No dilated biliary radicles.*
- *The spleen, gall bladder, kidneys, adrenal glands and pancreas are within normal.*
- *No significant lymph node enlargement.*
- *No ascites.*

OPINION:

- ***Minimally enlarged liver showing a small lobulated focal lesion at hepatic dome (segment VIII) with signal characteristics and dynamic post-contrast enhancement pattern as described, suggestive of a small cavernous hemangioma rather than other diagnostic possibilities.***

YOURS SINCERELY,
DR. RAMY EL-TOUKHY, MD
LECTURER OF RADIO-DIAGNOSIS
CAIRO UNIVERSITY

٢٣٥٨٧٣٢١ - ٢٣٥٨٨٨١٩ - ٠١١٠٠١٨١٢٨٠

M.R.I
[[DOCTYPE:MRI]]

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Radiology Department

وزارة الصحة والسكان
الهيئة العامة للمراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Marwa Abd Al Fattah Abd Al Rahman Ahmed	Date : 27/02/2025
Exam : C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 3118/2022	Pat. ID: 16757

- No adnexal or uterine mass with IUCD.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- **Rather stationary course of the** previously reported axial and appendicular skeleton widespread sclerotic foci yet developing more lytic texture of some of the previously noted osseous lesions, most evident at D12 level.

Opinion:

Post left MRM follow up revealed:

- **Currently appreciated small hepatic focal lesion (in current contrast enhanced study) for follow up**
- **Overall stationary course.**

Dr. Mohamed Mohsen, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name	: مروة عبدالفتاح عبدالرحمن احمد / مروة عبدالفتاح عبدالرحمن احمد	Request No.	: 13001865
Patient No.	: 27902170102521	Request Date	: 24/01/2024
Path No.	: 350/24	Verification Date	: 04/02/2024
Sex	: Female	Financial Category	: برنامج دار السلام - فئة الدولة
Age	: 44 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left skin thickening after MRM.
Nature of Specimen: Tru-cut biopsy, collected at 23/1/2024.

Technique:
Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
IPT (Positive control showed appropriate reactivity):
E-Cadherin: Negative.
ER: No nuclear reaction in the tumor cells (Allred score 0/8).
PR: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
Her2: Faint incomplete membrane reaction in >10% of the tumor cells (score1+).

Conclusion:
Left skin thickening after MRM, Tru-cut biopsy:
Invasive lobular carcinoma, Grade II.

E-Cadherin: Negative.
ER: Positive (score 8/8).
PR: Negative (score 0/8).
Her2: Score 1+.

Verified by :
Dr.Marwa mahmoud

Room No. (4) - First Floor-Building No. 987- Nile Cornishe - Misr El Qadema - Cairo, Egypt
غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	: الاسكلاء / مروة عبدالفتاح عبدالرحمن احمد	Request No.	: 13001865
Patient No.	: 27902170102521	Request Date	: 24/01/2024
Path No.	: 350/24	Verification Date	: 29/01/2024
Sex	: Female	Financial Category	: ار السلام - بقة الدولة
Age	: 44 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Ali mostafa

Pathology Report

Clinical Data: Left skin thickening after MRM.

Nature of the specimen: Tru-cut biopsy, collected at 23/1/2024.

Gross Examination:
Two gralysh white tissue cores measured up to 1 cm in length, totally submitted.

Microscopic examination:
- Histological type: Deposits of invasive mammary carcinoma

Diagnosis:
Left skin thickening after MRM; Tru-cut biopsy:
- Deposits of invasive mammary carcinoma
- Case is for IPT (E-cadherin)

Prof.Dr. Lobna Omar
Dr.Menna Nabil

Verified by :
walaa ghoniem

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
مدينة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Manwa Abd Al Fattah Abd Al Rahman Ahmed	Date : 27/02/2025
Exam : C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 31 18/2022	Pat. ID: 16757

- No adnexal or uterine mass with IUCD.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- **Rather stationary course of the** previously reported axial and appendicular skeleton widespread sclerotic foci yet developing more lytic texture of some of the previously noted osseous lesions, most evident at D12 level.

Opinion:

Post left MRM follow up revealed:

- **Currently appreciated small hepatic focal lesion (in current contrast enhanced study) for follow up**
- **Overall stationary course.**

Dr. Mohamed Mohsen, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]